SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Weekly cetuximab + FOLFIRI

Advanced / metastatic colorectal cancer · NCCP 00328 · version 7 · 14-day cycle

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Patient name	DOB	Hospital no.	Date

Treatment context
The aim is to control advanced bowel cancer and symptoms. The team reviews how well treatment is working and whether side effects remain acceptable; there is no single fixed number of cycles for everyone.

Schedule
Cetuximab on Day 1 and Day 8. Day 1 FOLFIRI includes irinotecan, folinic acid and fluorouracil, followed by a 46-hour fluorouracil infusion with planned disconnection around Day 3. Repeat the reference cycle every 14 days while beneficial and tolerable.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ DPD result and severe fluoropyrimidine risks reviewed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Cetuximab
EGFR-TARGETED ANTIBODY

- ☐ Rash, dry skin, cracks and sore nails
- ☐ Eye inflammation or corneal injury
- ☐ Low magnesium, calcium or potassium
- ☐ Diarrhoea, dehydration and kidney problems
- ☐ Infusion reaction; rare serious lung or skin effects

Irinotecan
TOPOISOMERASE I CHEMOTHERAPY

- ☐ Early cramps, sweating and diarrhoea
- ☐ Delayed diarrhoea and dehydration
- ☐ Low blood counts / serious infection
- ☐ Nausea, mouth soreness and fatigue
- ☐ Hair thinning or loss
- ☐ Rare serious lung or allergic reactions

Folinic acid
CHEMOTHERAPY MODULATOR

- ☐ Enhances fluorouracil activity and toxicity
- ☐ Rare allergic reaction
- ☐ Diarrhoea / mouth soreness in combination

Fluorouracil
CHEMOTHERAPY

- ☐ Diarrhoea, dehydration and mouth ulcers
- ☐ Low blood counts / infection
- ☐ Hand-foot skin soreness
- ☐ Cardiotoxicity including chest pain / ischaemia
- ☐ Severe early toxicity including DPD-related toxicity
- ☐ Pump / central-line complications

Discuss the treatment aim, alternatives, expected benefit, serious and lasting risks, fertility/pregnancy precautions and the contact plan. Confirm RAS selection for colorectal treatment, infusion premedication, skin care and post-treatment mineral checks. Explain DPD testing and the exact pump schedule. Explain early and delayed diarrhoea separately. Discuss severe or delayed infusion reactions and the agreed observation plan; a previous severe cetuximab reaction must be reported even after recovery. Explain that folinic acid supports fluorouracil and follows its bolus hold/omission plan; DPD testing, heart symptoms and current/recent interacting medicines can change whether fluorouracil is given. Discuss how previous severe diarrhoea, low blood counts, bilirubin and known UGT1A1 results may change irinotecan dosing and monitoring.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00328 v7.

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